## *Point-of-Care Testing*

***Point-of-care testing (POCT)*** is when labs are performed at the location where the patient is at, such as at the bedside. This is often done using portable devices such as a glucometer to check a patient's blood sugar (glucose levels). This allows for convenience for the patient and quick results for the patient's care team. There must be protocols in place to ensure the POCT is done correctly and that the results are the same as they would be if the test were performed in the laboratory. POCT requires QCs to be performed to ensure that this is the case. Follow the manufacturer's directions for how to complete QCs on POCT devices. Manufacturers of POCT devices have begun adding internal quality controls to their devices as another measure of ensuring accuracy.

*point-of-care testing (POCT).* Clinical laboratory testing that is done close to the location where patient care and treatment are provided.

## *CLIA-Waived Tests*

Three testing categories are designated based on their complexity: waived, moderate, and high complexity. These categories come from the Clinical Laboratory Improvement Amendments of 1988 (CLIA '88). All laboratories that are subject to the regulations of CLIA '88 are required to obtain a certificate according to the complexity of the labs performed at their facility. The complexity of testing is based on the difficulty involved in performing each specific test and the degree of harm that can be caused to a person or patient if the test is not performed correctly. The higher the risk or degree of harm, the higher the complexity of the test is. CLIA–waived tests have the lowest complexity. These tests are simple and have a low risk of harming a patient should the results be inaccurate. Laboratories that perform CLIA–waived tests require a Certificate of Waiver. The most common CLIA–waived test the phlebotomist performs is a blood glucose level. This can be done using a portable glucometer. The QC testing on a glucometer is performed using two of the liquid controls provided by the manufacturer. The results from the liquid controls are then compared against the target ranges provided by the manufacturer. After QCs are performed, they must be documented in the logbook. If QCs come back within the target range, the glucometer can be used for patient testing. If they are not within range, QCs should be rechecked. If they are still inaccurate at that time, new quality control reagents should be used. If the QCs are still inaccurate, the glucometer should not be used for patient testing and the manufacturer should be contacted. Once the glucometer QCs are done or a patient test is run, the glucometer should be cleaned or disinfected according to the manufacturer's guidelines, often with a disinfectant wipe.

### Testing Complexity

Phlebotomists are not able to run high-complexity testing. These can only be run by medical laboratory technicians or technologists with a minimum of an associate's degree and the proper training and licensure or certification.